

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	748
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	10/2011
		Review Date:	07/2028

OB-GYN EMERGENCIES

For Routine Newborn Delivery – **Refer to EMS Policy #700 – General Procedures.**

For Neonatal Resuscitation – **Refer to EMS Policy #749 – Newborn Resuscitation.**

Obtain pertinent history:

- Last menstrual period (LMP)
- Determine gestational weeks in the pregnancy
- Gravida (G) = how many pregnancies
- Para (P) = how many live births

BLS

Assess Vitals / Obtain SpO₂ / Oxygen - Titrate to SpO₂ of 94% or higher / Secure Airway - Assist Ventilations if needed.

Position – Position the mother in the left lateral position.

Premature Delivery

The infant may be delivered via Routine Delivery as stated above.

A premature infant often presents with an immature respiratory system - Be prepared to resuscitate the infant.

If non-viable, save and transport any tissue or fetal remains.

Prolapsed Cord

Position mother in knee to chest position or elevate pelvis with pillows or wedge device

- The head should be lower than the pelvis

Insert sterile gloved hand into vagina & gently push presenting part (head or shoulder) off the cord

- Do not tug on the cord
- Do not attempt to push the cord back in the birth canal
- Cover exposed portion of the cord with saline soaked gauze

Do not remove your hand until the hospital personnel assume care.

Breech Delivery – Buttocks Presentation

Allow the infant to deliver buttocks and trunk/waist spontaneously.

Once the legs are delivered, support the infants' body, and allow the head to deliver.

Gently rotate the baby to a face down position and continue with the delivery.

If the head does not deliver, insert sterile gloved hand into the vagina and make a "V" around the baby's

Signatures on File

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nose to push the birth canal away from the face to allow for ventilation

- This also takes some pressure off the cord

Breech Delivery – Limb Presentation

If foot (leg) or hand (arm) presentation:

- Place mother in knee to chest position or elevate pelvis with pillows or wedge device
- The head should be lower than the pelvis

It is unlikely that the infant will be delivered - Immediate transport should be initiated.

ALS

Follow BLS procedures if applicable.

Obtain 4-Lead.

Vascular Access – Establish large bore IV - IV/IO rate as appropriate.

Eclampsia with Seizures

Midazolam - 2 mg IV/IO OR 5 mg IN/IM

- Titrate IV/IO in **1 mg** increments for seizure control with a max dose 5 mg
- May repeat IN/IM once after 10 minutes if seizures continue

If the patient is more than 24 weeks pregnant:

Magnesium Sulfate - 2 gms in 10 ml NS slow IVP over 2 minutes